

Relevant Research

Losses Associated With Childhood Sexual Abuse in Women Survivors

Sexual abuse in childhood can leave a lasting impact on the victim. In a scoping review done by Kays Ebrahim et al. (2022), twenty studies were used to discuss the losses faced by women survivors. The four core losses are that of traumatic sexualization, stigmatization, betrayal, and powerlessness. However, a study by Bloom (1994) found that a new factor of loss associated with childhood sexual assault is worth studying. In general, most losses are associated with death, though any stressful occurrence can cause a loss of personal, interpersonal, symbolic, or psychological nature. Due to the stigma of CSA, these losses are typically not acknowledged, and can affect the way a victim processes their grief of the event. In an attempt to ease the suffering of the victim, it is important for treatment interventions to address these issues of stigmatized loss. The scoping review by Kays Ebrahim et al. (2022) found that female survivors of CSA can experience an overall loss of self efficacy evident by a loss of personal, interpersonal, and sexual agency. Female survivors may experience a loss of trust in themselves, leading to a lowered self-esteem and confidence, as well as an overall damage to self-identity. Victims may experience a loss of emotional control as a result of the trauma faced, and a loss of childhood innocence. Due to the majority of CSA cases being experienced by known perpetrators, many women may find that their loss of personal agency can be amplified by the deprivation of security and safety assumed of a childhood home. Female survivors may experience a disruptive influence in the development of their interpersonal skills. This can include a loss of social support, trust in others, and at times a loss of faith in religion. Following trauma caused by CSA, survivors can find trouble with trusting others. Especially if the abuser was a trusted party, they may begin to grieve the loss of this relationship, and find distrust in other interpersonal relationships following such loss.

Notes.

Client presented to the clinic with symptoms of PTSD, due to sexual abuse in childhood. The abuse was perpetuated by a close family member, and continued throughout adolescence

until the person was removed from the home. Following the removal of this person, the Caregivers began to notice behavioral concerns in Client, leading to the presenting problems seen during intake. At the start of treatment, the client still had phone interactions with this person, though highly monitored. Following an incident in one of these interactions, contact was completely severed and it was advised that Client no longer speak with the offender.

Focusing on Patients' Existing Resources and Strengths ...

When treating through Cognitive-Behavioral Therapy, it can be beneficial to focus on existing strengths and resources of the patient by following a capitalization model, as described by Cheavens et al.(2012). In a systematic review by Munder et al. (2019), available studies focusing on the use of strength-based and resource-focused adaptations of traditional CBT were examined for efficacy. Out of ten eligible studies, six used CBT as the base orientation, three used psychodynamic therapy (PDT), and one was conducted in an inpatient setting with mixed treatment backgrounds. Of these studies, eight were judged to have a high overall risk of bias, while two were judged unclear (all biases were due to non-random design). A small to moderate advantage of resource-focused therapy (RFT) was found, in comparison to alternative psychotherapies. The conclusion by Munder et al. (2019) was that hypnotherapeutic and systemic interventions are an effective add-on to CBT for patients with anxiety or depression, however further studies should be done to investigate the enduring effects of RFTs in post-traumatic stress patients. The results were inconclusive for that of imaginative techniques in PDT for post-traumatic stress patients.

Notes.

The clinician and supervising team developed a treatment plan for the client based on age and cognitive ability. The clinician then found ways to make the sessions more reinforcing for Client, as well as focus on the existing strengths and resources of their abilities. By ensuring sessions included positive reinforcement for completion of emotion regulation skills, as well as request Caregivers' assistance in homework tasks, it allowed for Client to gain the necessary skills required before moving on to trauma processing.

Trauma-Focused Cognitive Behavioral Therapy for Traumatized Children and Families

Two-thirds of American children and adolescents report at least one experienced trauma, with one-third experiencing multiple before reaching adulthood. This exposure is associated with an increased risk of medical and mental health problems, including PTSD, depression, anxiety, and suicidal attempts or completions. A study by Cohen and Mannarino (2015) looked at fifteen randomized controlled trials to examine the influence of parental inclusion in Trauma-Focused Cognitive Behavioral Therapy (TF-CBT). Parental influence is an important factor in childhood trauma response, and non-offending parents often become the child's primary source of safety and support. Often times, TF-CBT will include sessions with the parental figure, to update them on the skills being learned by the child, as well as motivate the parent to help the child in practicing these skills. These sessions allow for open communication between the clinician and the parent, and at times can allow the clinician to correct demeaning language or ineffective parenting skills being used with the child. It is important for the family to understand that TF-CBT is a collaborative treatment, therefore must include all parties to be willing and able to work with the therapist. Often, parents may disregard the positive behavior of the child, focusing only on the negative behaviors or responses, which reinforces these behaviors in the child. The conclusion by Cohen and Mannarino (2015) was that TF-CBT was found to be superior to that of the comparison condition, in regards to treating and improving symptoms of PTSD, and parental inclusion enhances and positively contributes to the outcomes of the child.

Notes.

Client was treated using the PRACTICE method of Trauma-Focused Cognitive Behavioral Therapy. While the initial treatment is beneficial to the treated individual, it is important to remember that the family is included in the well-being of the child, and may hold the primary influence over the lasting effect of the treatment. Including a time with Caregivers at the end of each session was beneficial to ensure that the entire family was on the same page in terms of treatment progress. This also allowed a safe space for Caregivers to discuss their frustrations, and gave the clinician a chance to address these concerns.

References

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